



WNHS PROCEDURE	
Safety Planning- WNHS Mental Health Service (MHS) Outpatient and Consultation Liaison (OPCL)	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	WNHS Mental Health Service (MHS) Outpatient and Consultation Liaison (OPCL)
This document should be read in conjunction with the Disclaimer .	

1. Aim

The purpose of this guideline is to provide direction and information to all mental health clinical staff of Women's Health, Genetics and Mental Health (WHGMH) regarding the assessment and management of risk to patients, consistent with the WA Department of Health [Principles and Best Practice for the Care of People Who May Be Suicidal](#).

WNHS Mental Health Service Outpatient and Consultation Liaison (OPCL) core business is to provide inpatient and outpatient mental health services to patients of Women and Newborn Health Service (WNHS). Mental health risk management for the purposes of this Guideline encompasses the awareness, appropriate assessment, treatment and management of risk of harm to self or others encountered in the course of service provision in this work environment.

Disclaimer - The authors have written these guidelines in good faith, hoping to assist in reducing risk for patients in Mental Health clinical areas. However, it is acknowledged that many leading authorities recognise that there is no evidence to support the notion that any risk assessment tool is effective in predicting those at risk of suicide or deliberate self-harm (See [Principles and Best Practice for the Care of People Who May Be Suicidal](#))

2. Risk

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Mandatory Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☐	Impact on Staff Safety	☒

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Other	☐		

3. Definition

Ensure all clinicians understand the principles of “Clinical Risk Identification” and have a common language when discussing clinical risk.

- Risk - The likelihood of an adverse event or outcome.
- Risk factors - Particular features of illness, behaviour or circumstances that alone, or in combination, affect levels of risk.
- No significant risk identified - Patient is unlikely to injure self or others, based on knowledge of behaviour, their mental health history, current stressors in person’s life, current mental and medical status and other risk factors.
- Significant risk - Patient may injure self or others, requires ongoing risk plan, regular review of mental status, including impulse control regarding harm to self or others; current stressors in person’s life; other risk factors identified.
- “Critical Points” – Points of patient treatment/care/intervention at which risk identification will occur:
 - first contact with service
 - assessments- initial and review
 - change or transfer of care
 - change in legal status
 - change in life event (e.g. loss, crises)
 - significant change in mental status
 - significant changes in supports (formal & informal)
 - discharge or move to a less supportive environment
 - going on overnight leave or prolonged periods of leave.
- Low risk - No known current or history of aggression, impulsivity, substance misuse, social stressors or instability.
- High risk - History of aggression, extenuating social situation, and threats of harm to self/others, family history of domestic violence, substance abuse associated with violence, history of mental illness associated with aggressive behaviour, aggressive partners, family, peers, housemates or pets.

4. Triage process and patient risk

- Two fulltime triage officers are assigned to respond to and manage patient referrals, related calls from patients, carers or referrers including regarding patient risk concerns.
- Patient referrals to OPCL via the appropriate Referral form have urgency levels, impact on functioning, Edinburgh Postnatal Depression Scale (EPDS)

suicidality screen, mental health diagnoses, history of mental health diagnoses and medication sections for completion by the referrer which gives some initial indication of levels of risk.

- Once referral is received, it is triaged by experienced mental health clinicians according to standardised PSOLIS/ WebPSOLIS online forms which include risk assessment and completion of the Risk Assessment and Management Plan (RAMP)
- Past PSOLIS Triage risk assessments, alerts and management plans are available via the PSOLIS mental health data base to authorised clinicians involved in mental health treatment and care. This provides historical risk related information regarding patients to subsequent services able to access PSOLIS.
- Triaged inpatient and outpatient referrals are presented to Multi-Disciplinary Team (MDT) reviews daily and reviewed, including for management of risk and devising further treatment pathway.
- Aggression alerts are placed on patient PSOLIS records where there is a history of, and ongoing risk of harm to self or others.
- Patients who cannot be contacted are sent a letter informing them of the referral to OPCL and requesting that they contact the Service.
- Medical charts of referred patients who are not able to be contacted are flagged for clinical review of determinable risk factors by Triage clinicians within 4 weeks of referral and either closed off if deemed low risk or efforts are renewed to contact the patient if risk factors are ongoing and significant.
- Referrers are informed of the outcomes of a referral in written form via medical chart communication, letter or email.
- Patients who present on an ad hoc basis, to the service during office hours, outside of the usual referral processes are reviewed by the Triage officer for risk and assessed if urgent by a Medical Officer or Consultant Psychiatrist.

5. Management of patient related risk including out of office hours

- On-call Psychiatry is available via the KEMH switch board for after- hour's urgent inpatient or Emergency Centre (EC) mental health risk assessment and management.
- Response for all referred patients includes, where indicated and as required, management of risk of harm to self or others assessed as acute or severe via administration of the [Mental Health Act 2014](#).
- The OPCL out of hour's phone answering message gives patients who contact and are experiencing a crisis, 24-hour crisis response numbers and directs them to their local major Hospital Emergency Department.

- Assistance is provided to the KEMH to manage mental health risk in patients according to triaged urgency including providing Psychiatric one to one nursing specials for inpatients, risk management plans, input into complex care patient planning and response and forming of patients under the Mental Health Act 2014.
- Consent from the patient for an assessment is sought by the referrer and preferred but not essential where there is significant clinical risk (see Mental Health Act 2014).
- All initial, review and final assessments of patients include a risk assessment. The mandatory standardised Mental Health Risk Assessment and Management Plan (RAMP) ([SMHMR905](#)) is used at initial assessment and at any further point during contact with DPM where significant risk levels are suspected (based on clinical judgement).
- Frequency of patient reviews is increased according to risk levels assessed as part of patient management plans. Patient diagnoses, treatment and management plans are documented on State-wide Standardised Clinical Documentation (SSCD) Assessment forms ([SMHMR902](#)) and Mental Health Care Transfer Summary forms ([SMHMR916](#)) and stored on the Patient's Medical record.

6. Staff Safety

- Duress alarms are installed in all clinician's offices and the front reception of OPCL, as well as hospital wards and clinics, in case of patient/other aggression. These are linked to hospital security response.
- All staff complete required mandatory aggression awareness and response online and face to face training including Code Black notification and response.
- CIMS (patient clinical incidents) and OSH hazard (staff critical incidents) processes may be relevant to risk management and are used as required.
- Professional supervision, peer support, debriefing and employee assistance program (EAP) access are routinely provided for OPCL staff responding to critical incidents involving patient/other risk management and adverse events.
- Complex patient treatment and management issues are reviewed in MDT clinical review meetings to support staff to maintain best practice principles in the management of patient risk issues.
- Staff can request assistance from hospital security staff when dealing with a high-risk situation.

7. Patient Risk Assessment

Risk may be conceptualised along a severity continuum of none, low, moderate, high or severe and as impacted upon by a number of exacerbating and mitigating or

protective factors which need to be assessed to determine level of risk and inform response/ management actions.

Severity criteria assessed include:

- significant life stressors and events experienced and perceived by the person as overwhelming, not able to be coped with and without prospect of adequate resolution
- history of personal and/or family adversity e.g. trauma, disruption, abuse, neglect and vulnerability
- current and or history of personal and family mental health problems
- current or recent substance use or abuse
- current significant problematic living conditions/circumstances
- history, presence, frequency and recency of occurrence of thoughts or actions of self or other harm. These may include at the less severe end of the spectrum, thoughts or images of an escapist nature e.g. going away on a long trip, imagining a different life. At the more severe end, this may present as recent and/or repeated history of serious harm behaviour without significant positive change in life situation or context. This is to be distinguished from non-suicidal self-harming behaviour with other functions e.g. superficial cutting to regulate distressing emotional responses.
- high level of distress and hopeless in presentation
- ability to articulate details of actions and methods of harm
- access to means of harm
- potential lethality of means articulated, including timing and multiple methods
- expressed intent to carry out self-harm behaviours
- refusal to disclose regarding means after indicating intent or to engage in planning for immediate personal safety
- distancing from formerly valued others, activities and possessions, including cognitive e.g. “they would be better off without me”.

Protective factors assessed include:

- accessibility and active presence of sufficient emotionally and practically supportive others and resources
- personal resilience in the face of adversity and self-efficacy regarding ability to cope
- range of effective coping strategies
- resolution of problematic stressors/events
- intelligence and emotional flexibility to adapt
- values and/or religious beliefs against self-harm
- engagement or willingness to engage in effective help seeking

- important and valued relationships
- sense of purpose in life and personal value to others
- spiritual/other belief system providing greater overview and solace.

8. Risk Management Planning

If the risk is current, the clinician in consultation with the patient (and/or significant other) provides recommendations and planning which may include:

- a recommendation to extend hospitalisation, pending further monitoring of safety and assessment of the patient
- consultation with senior Mental Health colleagues and a referral for further assessment
- organising for the patient to be transferred (voluntarily or as an involuntary patient if required under the Mental Health Act 2014) to an appropriate mental health facility such as the Mother and Baby Unit or an Adult Psychiatric facility
- a referral to and liaison with Social Work at KEMH if not already aware of and involved with the patient where there are concerns noted for the safety of the baby or the patient from domestic violence or other social circumstances of concern (alcohol or illicit substance use, intellectual or physical disability, homelessness, legal issues, financial difficulties, adolescent, minimal social supports etc.)
- organising ongoing out -patient OPCL clinic follow-up and monitoring or appropriate referral of the patient
- consultation and liaison with the Patient's GP and/or identified personal and professional carer/supports as clinically indicated and where possible, with the patient's consent with the aim of maintaining patient safety.

9. Recording

- A summary of the risk assessment and management plan is recorded in the Patient's Medical Record either in written format including formulation, mental status, risk factors, psychiatric diagnoses, follow-up and management plans or using the Mental Health Risk Assessment and Management Plan (RAMP) ([SMHMR905](#)).
- Medications are also noted and may require review by Psychiatric staff.
- Clinicians record each contact with the patient on the day it occurs in the integrated progressive notes.
- Clearance for discharge from an OPCL perspective and plans for ongoing monitoring, referral, or discharge are clearly documented and updated at the end of each summary entry.

Related policies

[Chief Psychiatrist's Standards for Clinical Care](#)

[Mental Health Act 2014](#)

[NMHS MH Clinical Risk Assessment and Management \(CRAM\) Policy](#)

[Principles and Best Practice for the Clinical Care of People Who May Be Suicidal](#)

[WA Health Mandatory Policy MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)

[WNHS Mental Health Policy for Women and Newborn Health Service \(WNHS\)](#)

Related WNHS procedures and guidelines

[Women's and Perinatal Mental Health Referral and Management Guideline](#)

Useful resources (including related forms)

[Statewide Standardised Clinical Documentation \(SSCD\) - Resources](#)

Keywords	Clinical handover, psychological medicine, mental health, procedure				
Policy Sponsor	Head of Department – WNHS Mental Health Service Outpatient and Consultation Liaison (OPCL)				
Policy Author	Consultant Clinical Psychology Director – WNHS Mental Health Service Outpatient and Consultation Liaison (OPCL)				
Date First Issued:	01/08/2021	Last Reviewed:	June 2025	Review Date:	June 2027
Approved by:	Mental Health Clinical Practice and Outcomes Committee			Date:	23/06/2025
Endorsed by:	Mental Health Clinical Practice and Outcomes Committee			Date:	23/06/2025
NSQHS Standards Applicable:	 Std 1: Clinical Governance  Std 5: Comprehensive Care  Std 6: Communicating for Safety  Std 8: Recognising and Responding to Acute Deterioration				
Chief Psychiatrist's Standards for Clinical Care		Risk Assessment and Management			
Printed or personally saved electronic copies of this document are considered uncontrolled					

Safety Planning –WNHS Mental Health Service Outpatient and Consultation Liaison (OPCL)

Version History

Version number	Date	Summary
1.0	February 2003	KEMH DC 07: Outpatient Risk Assessment and Review
1.1	July 2012, July 2017	Regular review and update (Other dates unknown).
2.0	August 2017	Merge of Psychological Medicine and MBU guidelines. Title: Clinical Practice Guideline –Risk Management - Mental Health. Review dates unknown
3.0	August 2021	Risk Management – Mental Health rescinded. New format- WNHS Procedure: Risk Management – Department of Psychological Medicine
4.0	June 2025	Three yearly review- minor amendments include: • title change to WNHS procedure: Safety Planning - WNHS Mental Health Service (MHS) Outpatient and Consultation Liaison (OPCL) to align with WA Health Mandatory Policy MP 0181/24 Safety Planning for Mental Health Consumers Policy. References to “Psychological Medicine” and abbreviations changed to reflect new departmental/service name change. • Hyperlinks updated • related policies added or updated • minor formatting changes which include removal of National Standards for Mental Health Service and replaced with Chief Psychiatrist’s Standards for Clinical Care to align with NMHS Mental Health

This document can be made available in alternative formats on request for a person with a disability.

© Women and Newborn Health Service 2025

Copyright of this material is vested in the State of Western Australia unless otherwise indicated.

Apart from any fair dealing for the purposes of private study, research, criticism or review, as permitted under the provisions of the Copyright Act 1968, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.